



THE DEMENTIA HOUSE CALL

A STAGE-BY-STAGE GUIDE FOR FAMILIES

The Middle-Stage Survival Guide

What to do when dementia gets hard — the daily battles, in plain language

Plain, practical guidance from a doctor who makes dementia house calls

Educational information for caregivers — not medical advice.

HOW TO USE THIS GUIDE

Read only the page you need

Every daily-battle page follows the same shape, so you can find what you need fast when it's happening:

- **What's happening** — why, in about thirty seconds.
- **In the moment** — what to try right now.
- **What makes it worse** — what to stop doing.
- **When it's not just the dementia** — the sudden changes that mean you call, not manage.
- **Borrow this sentence** — words that tend to work.
- **One line for the log** — the one note that finds the pattern.

THE ONE RULE THAT RUNS THROUGH EVERYTHING

A **sudden** change — new confusion, agitation, or sleepiness over hours or a day or two, often with a fever, pain, a change in urine, or after a new medication — is **medical until proven otherwise**. That is usually delirium, it is frequently reversible, and it needs a phone call, not a behaviour strategy. When something changes fast, think medical first.

A NOTE ON THE FOOTNOTES

A few pages point to products or services that genuinely help at this stage. Those are honest suggestions, not prescriptions — use only what helps, always with your person's knowledge. Some are affiliate links, which means The Dementia House Call may earn a small commission at no extra cost to you; it never changes the guidance, and we never steer a medical decision toward a purchase.

WHERE YOU ARE NOW

Where you are now

If you are reading this, the easy questions are behind you. Your person can still walk, still eat, still knows your face — but the days have gotten hard in a way the early ones weren't. They need help now with the private things: the shirt buttons, the shower, the bathroom. The afternoons come apart. They ask the same thing on a loop, or they turn on the person who does the most for them — which is usually you.

Clinicians call this stretch **moderately severe dementia**, or **Stage 6** on a scale called FAST. You don't need the label. What you need to know is this: **almost everything frightening you right now is the disease doing exactly what this stage does.** It is not you failing. It is not them being difficult on purpose. It is a brain losing the parts that plan, remember the last five minutes, and read a situation.

What is normal in this stage

- Needing help to dress, bathe, and use the toilet — usually in that order.
- Bladder and then bowel accidents, often later in the stage.
- **Sundowning** — agitation, pacing, or confusion that builds in the late afternoon and evening.
- Suspicion and accusations: hidden money, a "stolen" purse, a spouse who is "an impostor."
- Repeating the same question or story minutes apart.
- Wanting to "go home" while sitting in the home they've lived in for forty years.
- Sleep turned upside down.
- Still recognizing faces, but muddling names and relationships.

What is not just the dementia — and needs a call

A **sudden** change is a different animal from the slow slide of the disease. New confusion over hours or a day or two, a fever, a change in urine, new pain, a fall, a medication just started — any of these can tip a person with dementia into **delirium**, which looks like "the dementia got much worse overnight." It is often reversible, and it is a medical problem, not a behaviour to manage. When something changes *fast*, think **medical first**. (There's a one-page recap on page 17, and the traffic-light guide in your Care Binder walks it through.)

What this guide will and won't do

It won't cure a single one of these moments. What it will do is hand you, for each one, the same four things a good house call gives a family: *why it's happening, what to try right now, what to stop doing because it's making it worse, and when to stop managing and pick up the phone.* Keep it where the hard moments happen — the kitchen, the bathroom door, the car. Read only the page you need.

You are doing one of the hardest jobs there is. Let's make the next hour easier.

SUNDOWNING

Sundowning

The late-afternoon unravelling

What's happening. As the light drops, so does your person's reserve. A brain tired from a day of hard work — of holding together a world that no longer makes sense — runs out of the fuel it uses to stay calm and oriented. Add the day's fatigue, low light and long shadows, hunger, a full bladder, or too little to do, and the late afternoon becomes the hardest hours. This is called sundowning. It is real, it is common in this stage, and it is not defiance.

IN THE MOMENT

1. **Lower the demands, not just the lights.** Stop asking questions, stop teaching, stop correcting. This is not the hour for the shower or the visit from the grandchildren.
2. **Turn the lights up before dusk, not after.** Close the curtains against the shadows and switch on lamps *early* — beat the change, don't chase it.
3. **Meet the body first.** Offer the toilet, a snack, a drink of water. A surprising amount of "agitation" is a full bladder or an empty stomach the person can no longer name.
4. **Go simple and rhythmic.** A folded pile of towels to sort, a familiar record, a slow walk down the hall, a warm drink. Repetition soothes.
5. **Lower your own voice and slow down.** They will borrow your nervous system. If you are calm and unhurried, some of that crosses over.

WHAT MAKES IT WORSE

- Reasoning, arguing, or reality-checking ("Dad, it's your house, you *are* home").
- A loud, busy, cluttered room; the TV news; several people talking.
- Caffeine or a big sugary snack in the afternoon; a long daytime nap.
- Scheduling anything hard — bathing, appointments, outings — for late in the day.

WHEN IT'S NOT JUST THE DEMENTIA

When it's not just the dementia. Sundowning builds slowly over the *same* hours most days. If confusion or agitation arrives **suddenly**, is far worse than usual, comes with a fever, pain, a change in urine (smell, colour, burning), or after a new medication — that's a flag for **delirium or infection**, not sundowning. Call the doctor's office the same day; if there's chest pain, a hard fall, or they can't be roused, that's a 911 page. (*Traffic-light recap*, p.17.)

BORROW THIS SENTENCE

"You're safe, I'm right here, and we don't have to do anything right now." — then stop talking.

ONE LINE FOR THE LOG *time it started · what was happening just before · what helped.* Three days of that and the pattern — and the trigger — usually shows itself.

THE BATH / SHOWER REFUSAL

The bath that becomes a battle

What's happening. Bathing asks a person to undress in front of someone, step onto a slippery surface, and be hit with loud water and cold air — while no longer understanding *why*. In this stage the shower can feel frightening, exposing, even threatening. Refusal is usually fear or a bid for dignity, not stubbornness. (Difficulty bathing is such a hallmark of this stage that clinicians mark it as its own step, **FAST 6b**.)

IN THE MOMENT

1. **Drop the frequency.** Nobody in this stage needs a daily shower. Twice a week, plus daily "top-and-tail" washing of face, hands, underarms and groin, is clean and healthy. Winning the war can mean skipping the battle.
2. **Warm everything first.** Heat the room, hang a warm towel, run the water before they come in. Cold is a trigger.
3. **Cover as you go.** A towel or robe over the shoulders during a shower preserves dignity and warmth. Wash under it, section by section.
4. **Offer, don't announce.** "Let's get you freshened up before lunch" lands better than "It's time for your shower." Give a small real choice: bath or shower, now or after tea, blue towel or white.
5. **Consider a no-shower wash.** A warm bed-bath with rinse-free cloths, or a hairdresser sink for hair, gets the job done without the shower at all.
6. **Protect against falls.** A shower chair, a hand-held head, grab bars, a non-slip mat. Fear of falling is often *rational* — make it genuinely safer and some resistance melts.

WHAT MAKES IT WORSE

- Surprising them, rushing, or arguing about the last time they washed.
- Standing them up in cold air, naked, waiting for water to warm.
- Making it a test of hygiene or a point of principle. It isn't worth the trust it costs.

WHEN IT'S NOT JUST THE DEMENTIA

When it's not just the dementia. New, sudden resistance from someone who used to accept bathing — especially with grimacing, guarding, or a cry when you touch a shoulder, hip or back — can be **pain** (arthritis, an injury, a pressure area) they can't put into words. New agitation plus a change in urine can be a **UTI**. Both are worth a call, not just a workaround.

BORROW THIS SENTENCE

"I'll keep you warm and I won't let you fall. We'll go slow, and we can stop any time."

ONE LINE FOR THE LOG what you tried · what they said or did · what worked. Carry the winning method forward; consistency is half the battle.

When it turns to pushing, hitting, or shouting

What's happening. Aggression in dementia is almost always **communication of last resort** — the response of a frightened, cornered, or overwhelmed person who has lost the words to say *stop, I'm scared, that hurts, I don't understand*. It frequently erupts around the intimate-care moments (dressing, bathing, toileting) precisely because those feel most threatening. It is rarely about you, even when it is aimed at you.

IN THE MOMENT — SAFETY FIRST

1. **Give space.** Step back out of arm's reach. Don't corner them, don't loom, don't grab — even to "help." Most episodes cool faster with distance.
2. **Stop the activity.** Whatever you were doing, stop. The shirt, the shower, the pill can all wait ten minutes. Withdrawing the demand is often the whole fix.
3. **Lower and slow your voice.** Fewer words. "Okay. We'll stop. You're alright." Match their world's emotion, not its facts.
4. **Look for the cause behind the flare:** pain, a full bladder, hunger, too much noise, a stranger, being rushed, a task that overwhelmed them. Fix the cause and the behaviour usually settles.
5. **Come back later, changed.** Try again in 15–20 minutes with a different approach, a different person, or simply a different sentence. A fresh start often works where pushing never would.

WHAT MAKES IT WORSE

- Arguing, correcting, raising your voice, or "you always do this."
- Physically forcing or restraining a task through resistance.
- Too many people, too many instructions, too fast.
- Taking it personally in the moment (save that for later — it's allowed, but not here).

WHEN IT'S NOT JUST THE DEMENTIA

When it's not just the dementia — and this one matters. A **sudden** new aggression or agitation, unlike the person's baseline, is a red flag for **delirium, infection (especially urinary), pain, constipation, or a medication effect** — treat new agitation as possibly medical until proven otherwise, and call. If anyone's safety is genuinely at risk and you can't de-escalate, it is okay to leave the room, ensure the exits and hazards are safe, and call for help. Keep a plan for where *you* can go.

A WORD ON MEDICATION

Sedatives and antipsychotics are **not** the first tool for these behaviours. In older adults with dementia, antipsychotics carry a boxed warning for increased risk of death and stroke, and are reserved for when there is a real danger and non-drug approaches have genuinely failed — started low, reviewed often, and always a conversation with the prescriber, never a default. (*More on p.18.*)

BORROW THIS SENTENCE

"I'm sorry — let's stop. You're safe. I'll give you a minute."

ONE LINE FOR THE LOG *what happened right before · what the behaviour was · what calmed it.* The trigger is the treatment; the log is how you find it.

GETTING DRESSED (FAST 6A)

The clothes fight

What's happening. Getting dressed is a long chain of small decisions — what to wear, which way round, which arm first, buttons, left shoe or right. A brain that can no longer hold a sequence gets lost partway, and the whole thing stalls into frustration. It is also intimate, and being undressed and handled can feel like a loss of control. Needing help to dress is usually the first of the "hands-on" steps clinicians mark in this stage (**FAST 6a**).

IN THE MOMENT

1. **Lay it out in order.** One garment at a time, in the sequence it goes on, facing the right way. Hand them the next piece only when the last is done.
2. **Shrink the choice.** Two tops, not a full closet. Or none — just present the outfit.
3. **Change the clothes, not the person.** Elastic waists, slip-on shoes, front-openings, Velcro instead of buttons and zips. Adaptive clothing removes the fight entirely.
4. **Let them do what they still can,** even slowly. Doing it *for* them faster costs them a skill.
5. Warm room, privacy, no rush, gentle narration.

WHAT MAKES IT WORSE

- A closet full of choices; fiddly buttons, zips, and laces.
- Rushing, or silently doing it all for them.
- "You've got it on backwards again."

WHEN IT'S NOT JUST THE DEMENTIA

When it's not just the dementia. New refusal with wincing or guarding when you move a limb can be **pain** (a joint, a hidden injury). Sudden new clumsiness or one-sided weakness is a medical change — think stroke — and needs a call.

BORROW THIS SENTENCE

"Let's do this together — you start, and I'll help with the tricky bits."

ONE LINE FOR THE LOG *what made it easier (elastic waist, laying it out in order).*

"THEY WON'T EAT"

When they stop eating

What's happening. Appetite, smell and taste all change. They may not recognise food as food, forget how to use a fork, feel swamped by a full plate, have a sore mouth or loose dentures, or be badly constipated. Very little of this is stubbornness — it is the disease and the body getting in the way.

IN THE MOMENT

1. **Simplify the plate.** One food at a time, favourites, finger foods they can eat without cutlery.
2. **Small and often** beats three big meals. Calories over balance at this stage — full-fat, their favourites, a milkshake if that's what goes down.
3. **Check the mouth and the bowels.** A sore, thrush, ill-fitting dentures, or constipation will all shut appetite down — and all are fixable.
4. **Eat together.** Model it; put a fork in their hand; a calm, unhurried table.
5. Use a plain, contrasting plate so the food stands out, and the right texture — soft or minced if swallowing has become hard.

WHAT MAKES IT WORSE

- A big, full plate; pressure, coaxing, or arguing.
- Rushing, distractions, the TV blaring.
- Complicated cutlery.

WHEN IT'S NOT JUST THE DEMENTIA

When it's not just the dementia. **Coughing, choking, or a wet, gurgly voice during meals** is an aspiration risk — ask for a swallowing assessment. New refusal with fever, mouth pain, or holding food in the cheeks is medical. Rapid weight loss deserves a review. *And a hard truth worth knowing early:* in **advanced** dementia, careful hand-feeding for comfort is kinder and safer than a feeding tube — tubes don't prolong life or prevent pneumonia here. (This is covered fully in the site's eating letter.)

BORROW THIS SENTENCE

"You don't have to finish it — just a few bites for me."

ONE LINE FOR THE LOG *what and when they ate best.*

THE SAME QUESTION, AGAIN

The question on a loop

What's happening. They can't hold the answer for more than a moment, so the worry underneath it never gets put to rest. The repetition is almost always an **unmet feeling** — anxiety, needing reassurance, not knowing what happens next — wearing the disguise of a question. Answer the feeling and the loop usually loosens.

IN THE MOMENT

1. **Answer the feeling, not the fact.** "You're safe. I've got everything handled." Say it warmly, the same way each time.
2. **Keep it short and identical.** New words each time make it a new puzzle.
3. **Write down the recurring ones.** A whiteboard by the chair — "*Lunch is at 12. We are staying home today.*" — lets them check without asking.
4. **Redirect gently** to a small activity once you've reassured.
5. Never "*I just told you.*"

WHAT MAKES IT WORSE

- "You already asked me that." Irritation they can hear in your voice.
- Long, logical explanations. Quizzing them to "help their memory."

WHEN IT'S NOT JUST THE DEMENTIA

When it's not just the dementia. A sudden surge in repetitive questioning or agitation, different from the usual, can signal **delirium, pain, or a UTI** — worth a call.

BORROW THIS SENTENCE

"We're all set — there's nothing for you to worry about."

ONE LINE FOR THE LOG *what the question is really about — usually a worry about time, safety, or where someone is.*

SUSPICION, ACCUSATIONS & DELUSIONS

"You stole my purse"

What's happening. A brain that misplaces things and can't retrace its steps fills the gap with a story that fits — *someone took it*. Delusions — a hidden fortune, a stranger in the house, a spouse replaced by an impostor — are the same thing on a larger scale: the mind explaining a world it can no longer follow. **It feels completely real to them.** It is a symptom, not a lie, and not really about you — even when it lands on you, because you're the one who's there.

IN THE MOMENT

1. **Don't argue or defend.** You cannot win a fact-fight, and trying makes you the enemy. Step into their reality: *"That's upsetting — let's look for it together."*
2. **Help "find" it.** Keep duplicates of the usual suspects (glasses, purse, keys), and learn their hiding places.
3. **Reassure safety**, then redirect to something calming.
4. **Reduce triggers:** cover mirrors if their own reflection frightens them; turn off a TV they mistake for real people in the room.
5. Don't take it to heart in the moment — save that for later, with someone who gets it.

WHAT MAKES IT WORSE

- Arguing, proving them wrong, producing "evidence."
- Defensiveness, or laughing it off.

WHEN IT'S NOT JUST THE DEMENTIA

When it's not just the dementia. **New, sudden** paranoia or accusations — especially with fresh confusion — point to **delirium, infection, or a medication**, and need a call.

Delusions that terrify them or make them unsafe deserve a medical review; some are treatable (and see the medication note on p.18 — sedatives are a last resort, not a first).

BORROW THIS SENTENCE

"I can see you're upset — let's sort this out together."

ONE LINE FOR THE LOG *the accusation, what happened just before, and what settled it.*

SEEING THINGS THAT AREN'T THERE

Hallucinations

What's happening. The brain misfires and paints in sights — sometimes sounds — that aren't there. It's especially common in **Lewy body dementia**. Not all of it is frightening, and what doesn't frighten them may not need "fixing" at all.

IN THE MOMENT

1. **Don't argue it away, and don't pile on.** *"I don't see it — but I'm right here, and you're safe."*
2. **If it's gentle** (a child, a cat, a visitor), it's fine to go along kindly.
3. **If it frightens them**, reassure, turn the lights up, and remove the trigger — shadows, reflections, a busy patterned curtain, a coat on a door.
4. **Check glasses and hearing aids.** A blurry, muffled world invents things to fill the gaps. (Treating hearing and vision genuinely helps — see the early-stage guide.)
5. Your calm is the treatment; they borrow it.

WHAT MAKES IT WORSE

- Insisting it isn't real; arguing.
- Dim, shadowy rooms; sudden noises; clutter.

WHEN IT'S NOT JUST THE DEMENTIA

When it's not just the dementia. **New** hallucinations that arrive suddenly signal **delirium, infection, a medication, or an eye/ear change** — call. One caution: people with Lewy body dementia are **dangerously sensitive to antipsychotics**, so these are never a first move and always a medical decision.

BORROW THIS SENTENCE

"You're safe. I'm right here with you."

ONE LINE FOR THE LOG *time, what they saw, whether it frightened them, and the lighting.*

WANDERING & EXIT-SEEKING

The urge to leave

What's happening. Walking off usually has a reason that makes sense to them: looking for a child who's now grown, "going to work," heading "home," restlessness, boredom, needing the toilet, or pain. The behaviour is purposeful; the danger is getting lost or leaving into traffic or cold.

IN THE MOMENT (AND AHEAD OF IT)

1. **Meet the need first** — toilet, a snack, a walk, company, something to do. Much "wandering" is an errand the body can't name.
2. **Make the exit quiet.** A curtain over the door, locks above or below eye-line, a chime or sensor when the door opens.
3. **Plan for the worst calmly:** an ID bracelet, a recent photo, a wander-response service, and neighbours who know.
4. **Burn the restlessness off** — daytime activity and a real walk reduce the urge better than any lock.
5. Don't block or forbid — **walk *with them***, then loop gently back.

WHAT MAKES IT WORSE

- A hard "no," blocking the door, a physical tussle.
- Boredom, unmet needs, an over-stimulating or an under-stimulating day.

WHEN IT'S NOT JUST THE DEMENTIA

When it's not just the dementia. A sudden new restlessness or pacing can be **pain, constipation, a UTI, or delirium** talking — check before you manage.

BORROW THIS SENTENCE

"Let's get our coats and go together" — then walk, and turn back.

ONE LINE FOR THE LOG *when and where they head, and what they seem to be seeking.*

TOILETING & INCONTINENCE (FAST 6C–6E)

Without shame

What's happening. Continence goes in steps — first the *mechanics* (finding the toilet, clothing, the sequence), then bladder accidents, then bowel. This is the disease following its course (**FAST 6c–6e**), not laziness or spite. It is also, for most families, the hardest and most private part — so **dignity comes first, always.**

IN THE MOMENT

1. **Don't wait to be asked.** Offer the toilet on a schedule — every two to three hours, after meals, before bed.
2. **Signpost the way:** a picture on the door, a night-light, the door left open, a clear path.
3. **Watch for the tells** — pacing, tugging at clothing, restlessness — and act on them.
4. **Easy clothing** (elastic, Velcro) and well-fitted products, handled matter-of-factly.
5. **Protect the skin** — clean gently, use a barrier cream, check for redness (this matters more as they move less; the late-stage guide covers pressure areas).
6. Trim evening fluids a little — but **never** restrict daytime drinking; dehydration causes confusion, falls, and UTIs.

WHAT MAKES IT WORSE

- Any hint of shame or scolding. Waiting for them to ask.
- Hard-to-remove clothing; a dark or hidden bathroom; cutting daytime fluids.

WHEN IT'S NOT JUST THE DEMENTIA

When it's not just the dementia. A **sudden** change — new incontinence, cloudy or strong-smelling urine, burning, new confusion or agitation — is very often a **UTI**: extremely common, and very treatable. Call. A sudden change in bowels may be **constipation or impaction**, which can cause overflow leaks and real distress.

BORROW THIS SENTENCE

"Let's freshen up — this happens to everyone, no fuss at all."

ONE LINE FOR THE LOG *the time accidents tend to happen, and what came just before.*

NIGHT WAKING & THE FLIPPED CLOCK

When day and night change places

What's happening. Dementia damages the body's clock. Add too little daylight, daytime dozing, and not enough to do, and the day flips: they wake at 3 a.m. convinced it's morning, get dressed, or try to leave.

IN THE MOMENT (AND AHEAD OF IT)

1. **Anchor the morning** — bright light, a real walk, activity early. Keep daytime naps short.
2. **Wind the evening down** — dim lights, a calm routine, a comfortable temperature, a night-light, a clear path to the toilet.
3. **Toilet before bed**; ease off evening fluids and caffeine.
4. **If they wake, stay boring**. Low light, low voice, reassure it's night, guide them back. Don't switch the day on — no bright lights, no TV.
5. Rule out the simple things: pain, a full bladder, hunger, cold.

WHAT MAKES IT WORSE

- Long afternoon naps, caffeine, bright screens at night.
- Arguing about what time it is; a busy, stimulating evening.

WHEN IT'S NOT JUST THE DEMENTIA

When it's not just the dementia. A sudden new bout of night agitation or confusion can be **delirium, pain, or a UTI** — call.

BORROW THIS SENTENCE

"It's still night-time — I'm here. Let's tuck back in."

ONE LINE FOR THE LOG what time they wake, and what settled them.

"I WANT TO GO HOME" (WHEN THEY'RE ALREADY HOME)

The home they mean isn't a place

What's happening. "Home" is rarely the building. It's a *feeling* — safety, belonging, a time when the world made sense, often childhood. Asking to go home is asking to feel secure. Which is why *"but you ARE home"* never works: it argues with a feeling using a fact, and the feeling wins every time.

IN THE MOMENT

1. **Don't correct. Join.** *"Tell me about home — what's it like?"*
2. **Let them reminisce.** Talking about the home and people they mean is itself soothing, and it tells you what they're missing.
3. **Reassure, then redirect:** *"We'll head out a bit later — let's have a cup of tea first."*
4. **Build the feeling of home now** — a familiar blanket, a photo, a favourite song, a warm drink.
5. It's often worst in the late afternoon — reach for the sundowning tools (p.4).

WHAT MAKES IT WORSE

- "This is your home." Reasoning, proving, showing them the deed.
- Promising to take them "later" and then never — they may not remember the words, but they feel the letdown.

WHEN IT'S NOT JUST THE DEMENTIA

When it's not just the dementia. A sudden spike in this distress, well beyond the usual, can carry **pain or delirium** underneath — check.

BORROW THIS SENTENCE

"Tell me about home — what do you miss most?"

ONE LINE FOR THE LOG when it tends to start (often late afternoon), and what comforts them.

HOLDING IT TOGETHER

WHAT TO STOP WORRYING ABOUT

A few permissions, from a doctor, in writing:

- They **do not need a daily shower**. Twice a week plus a daily wipe is clean and healthy.
- A **kind untruth that comforts is kindness**, not lying. "Mum's just at the shops" beats the fresh grief of "Mum died in 1998," told again.
- **Dessert first is fine**. Mismatched clothes are fine. The battles that aren't about safety or comfort aren't worth the trust they cost.
- You **cannot reason them back into your reality**, and you don't have to. Meet them in theirs.

Save your energy for the two things that matter: **safety and comfort**.

WHEN A BEHAVIOUR IS ACTUALLY AN EMERGENCY (RECAP)

The single rule that runs through this whole guide: **a sudden change is medical until proven otherwise**. The disease moves slowly; infection and injury move fast. When confusion, agitation, or sleepiness arrives over hours or a day or two, look first for **a UTI, pneumonia, pain, constipation, dehydration, a new medication, or a fall/head knock** — this is **delirium**, and it's often reversible.

- **Call the office same-day** for a sudden change.
- **Call 911** for chest pain, a serious fall or head injury, a person you can't rouse, trouble breathing, or stroke signs (face droop, arm weakness, speech change).

(The traffic-light guide in the Care Binder walks this through, colour by colour.)

A WORD ON MEDICATION

The whole point of this guide is that **the first tools for these behaviours are not drugs** — they're the approaches on every page here. When medication is considered:

- **Antipsychotics carry a boxed warning** in older adults with dementia — an increased risk of stroke and death. They are reserved for genuine danger, after non-drug approaches have truly been tried, started low, and reviewed often. **Lewy body dementia is especially sensitive** to them.
- **Treat sudden new agitation as possibly medical first** — a UTI or pain is a kinder thing to find than a new sedative.
- **Fewer medicines often means fewer falls and less confusion**. Ask for a medication review (deprescribing) — the pharmacist and doctor can help.

Every one of these is a conversation with the prescriber, never a default.

YOUR OWN OXYGEN MASK

This stage can last years, and it asks more of you than any other. That is not a side note — it is the reason this page exists.

- **You cannot pour from an empty cup.** Accept help before you're desperate; use respite *early*, not as a last resort.
- **Guilt is normal.** It is not evidence you're doing badly — usually the opposite.
- **Grieving someone who is still here** (anticipatory grief) is real and allowed.
- Watch **your own** sleep, health, and mood. A support group or a counsellor helps more than white-knuckling it. (The site's "For you" letter and caregiver-support resources are there for exactly this.)

Asking for help is not failing. It's how the marathon gets run.

WHAT THE LATE STAGE LOOKS LIKE, AND HOW YOU'LL KNOW

So you're not blindsided: as the middle stage gives way to the late one, speech narrows to a few words and then to none; walking is lost, then sitting up, then swallowing; sleep takes up more of the day. And through all of it, **touch, a familiar voice, and music still land**. This is usually when comfort-focused care — and, often, a care home — become the kindest choices. It is not giving up. It is changing the goal to comfort and dignity. **The Late-Stage Guide picks up exactly here.**



THE DEMENTIA HOUSE CALL

You don't have to figure this out alone

The Dementia House Call sends free, plain-language letters for families — the same things a good house call would tell you. Join at **thedementiahousecall.com**, and look for the rest of the stage-by-stage series and the Care Binder.

The series: The Early-Stage Guide · The Middle-Stage Survival Guide · The Late-Stage Guide · and the Care Binder, the filing cabinet they all point back to.

This guide is general education for families and is not medical advice; it does not create a doctor–patient relationship and does not replace your own physician. Anything that looks like a sudden change, an infection, or an injury needs assessing, not managing — if you're unsure, assume it does and make the call. Some links in this guide are affiliate links; The Dementia House Call may earn a small commission at no cost to you, and it never changes the guidance. © 2026 The Dementia House Call.